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ZEPHYR PHARMA NIGERIA LIMITED

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Our Ref: ZPN/PV/PSUR/2026/014

Date: 12 September 2026

The Director-General

National Agency for Food and Drug Administration and Control (NAFDAC)

Pharmacovigilance and Post-Marketing Surveillance Directorate

Abuja, Nigeria

RE: SUBMISSION OF PERIODIC SAFETY UPDATE REPORT (PSUR)

We write to submit the Periodic Safety Update Report for the product detailed below, prepared in accordance with the NAFDAC PSUR Full Template and ICH E2C(R2).

Product name:	Amoxicillin/Clavulanate Potassium 500 mg/125 mg film-coated tablets
Active substance:	Amoxicillin (as trihydrate) / Clavulanate potassium
Strength(s) and dosage form(s):	500 mg/125 mg film-coated tablets; 875 mg/125 mg film-coated tablets; 250 mg/62.5 mg per 5 mL powder for oral suspension
NAFDAC Registration Number:	A4-100123
Renewal / application number:	REN/2026/04417
Reporting interval:	01 Sep 2025 – 31 Aug 2026
Data Lock Point (DLP):	31 August 2026
Marketing Authorisation Holder:	Zephyr Pharma Nigeria Limited, 14 Adeola Odeku Street, Victoria Island, Lagos, Nigeria

Qualified Person for Pharmacovigilance (Nigeria)

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This PSUR is submitted as **one report per active substance**, covering all registered strengths and dosage forms listed above. The product name and strengths are stated exactly as they appear on the

NAFDAC Certificate of Registration.

The reporting interval is **contiguous with the previous PSUR** submitted to NAFDAC, which covered 01 Sep 2024 – 31 Aug 2025 (Our Ref: ZPN/PV/PSUR/2025/011). There is no gap or overlap between the two intervals.

Queries and commitments raised in NAFDAC's assessment of the previous PSUR are addressed in Section 20 of this report.

Dr. Adaeze Nwosu

Qualified Person for Pharmacovigilance (Nigeria)

Zephyr Pharma Nigeria Limited

Date: 12 September 2026

PERIODIC SAFETY UPDATE REPORT

Prepared in accordance with the NAFDAC PSUR Full Template and ICH E2C(R2)

Product name	Amoxicillin/Clavulanate Potassium 500 mg/125 mg film-coated tablets
Active substance	Amoxicillin (as trihydrate) / Clavulanate potassium
Strengths and dosage forms covered	500 mg/125 mg tablets; 875 mg/125 mg tablets; 250 mg/62.5 mg per 5 mL oral suspension
NAFDAC Registration Number	A4-100123
Marketing Authorisation Holder	Zephyr Pharma Nigeria Limited, 14 Adeola Odeku Street, Victoria Island, Lagos, Nigeria
International Birth Date (IBD)	17 March 2014
Date of first NAFDAC registration	05 June 2018
Data Lock Point (DLP)	31 August 2026
Reporting interval	01 Sep 2025 – 31 Aug 2026
Report number in series	PSUR No. 9
QPPV (Nigeria)	Dr. Adaeze Nwosu · +234 803 400 1122 · adaeze.nwosu@zephyrpharma.example
Deputy QPPV	Pharm. Tunde Balogun · +234 805 900 4471 · tunde.balogun@zephyrpharma.example

Basis of the reporting interval. The reporting interval of this report is calculated from the **International Birth Date (17 March 2014)**, in accordance with ICH E2C(R2), and not from the date of first NAFDAC registration.

Confidentiality statement. This document contains confidential and proprietary information belonging to Zephyr Pharma Nigeria Limited. It is provided to NAFDAC solely for regulatory assessment and must not be disclosed to, or used by, any third party without the prior written consent of the Marketing Authorisation Holder.

Dr. Adaeze Nwosu, QPPV (Nigeria) — signed and dated 12 September 2026
Zephyr Pharma Nigeria Limited, 14 Adeola Odeku Street, Victoria Island, Lagos, Nigeria

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List of Abbreviations

ADR	Adverse Drug Reaction	MAH	Marketing Authorisation Holder
AE	Adverse Event	PSUR	Periodic Safety Update Report
DLP	Data Lock Point	QPPV	Qualified Person for Pharmacovigilance
IBD	International Birth Date	RMP	Risk Management Plan
ICSR	Individual Case Safety Report	RSI	Reference Safety Information
MedDRA	Medical Dictionary for Regulatory Activities	SmPC	Summary of Product Characteristics
SAE	Serious Adverse Event	PT	Preferred Term

Executive Summary

Introduction

This is the ninth Periodic Safety Update Report for Amoxicillin/Clavulanate Potassium film-coated tablets and oral suspension, prepared by Zephyr Pharma Nigeria Limited. It covers the interval **01 Sep 2025 – 31 Aug 2026**, with a Data Lock Point of **31 August 2026**. The interval is calculated from the International Birth Date of 17 March 2014 and is contiguous with the preceding report.

Marketing authorisation status

The product holds marketing authorisation in 14 countries. In Nigeria it is registered under NAFDAC Registration Number A4-100123, first registered on 05 June 2018. No authorisation was withdrawn, suspended or refused during the interval.

Estimated exposure

Estimated worldwide patient exposure during the interval was 412,000 patient-treatment courses, of which 38,400 were in Nigeria, derived from sales volume converted using the defined daily dose.

Safety data received

A total of 61 valid ICSRs were received, of which 9 were serious. No fatal cases were reported. The most frequently reported preferred terms were diarrhoea, nausea and rash, all of which are listed in the Reference Safety Information.

Signals and risks

One signal (cholestatic hepatitis with prolonged courses) was under evaluation at the start of the interval and was closed during it, with no change to the Reference Safety Information found to be warranted. No new signal was identified and no new risk was characterised.

Changes to the Reference Safety Information

No change was made to the Reference Safety Information during the interval. The current version, RSI v6.2 dated 14 January 2026, is attached as Appendix I.

Conclusions

The cumulative safety data reviewed in this interval are consistent with the established safety profile of the product. The benefit-risk balance for all approved indications remains **favourable and unchanged**. No regulatory action is proposed, and no change to the Reference Safety Information, the Risk Management Plan or the conditions of the marketing authorisation is required.

1. Introduction

This PSUR presents the worldwide safety experience for Amoxicillin/Clavulanate Potassium for the interval 01 Sep 2025 – 31 Aug 2026 (DLP 31 August 2026). It has been prepared according to the NAFDAC PSUR Full Template and ICH E2C(R2) and covers all registered strengths and dosage forms under NAFDAC Registration Number A4-100123. The approved indications are the treatment of bacterial infections of the upper and lower respiratory tract, urinary tract, skin and soft tissue, and of odontogenic infections.

2. Worldwide Marketing Authorisation Status

The product is authorised in 14 countries. The International Birth Date is 17 March 2014. First registration in Nigeria was granted on 05 June 2018. During this interval no new authorisation was granted, and none was withdrawn, suspended, revoked or refused.

3. Actions Taken in the Reporting Interval for Safety Reasons

No action was taken for safety reasons during the interval. Specifically: no product recall, no batch withdrawal, no suspension of supply, no clinical trial suspension, no dosage restriction, no change to the target population and no Direct Healthcare Professional Communication was issued in any territory.

4. Changes to Reference Safety Information

There was no change to the Reference Safety Information during the interval. The RSI in force throughout was **version 6.2, dated 14 January 2026**, attached in full as Appendix I. The Risk Management Plan in force is **RMP version 4.1**, approved 09 October 2025; no update was required during the interval.

5. Estimated Exposure and Use Patterns

5.1 Cumulative subject exposure in clinical trials

Cumulative exposure across the completed clinical development programme is 2,914 subjects. No trial was ongoing during this interval.

5.2 Cumulative and interval patient exposure from marketing experience

Region	Interval exposure (treatment courses)	Cumulative since IBD
Nigeria	38,400	247,900
Rest of Africa	96,100	612,400
Europe	184,300	1,408,700
Rest of world	93,200	733,500
Total	412,000	3,002,500

Exposure is estimated from sales volume converted using a defined daily dose of 1,500 mg amoxicillin equivalent over a mean course of seven days.

6. Data in Summary Tabulations

6.1 Reference information

MedDRA version 29.1 was used for coding throughout.

6.2 Cumulative summary tabulation of serious adverse events from clinical trials

No clinical trial was ongoing during the interval. The cumulative tabulation from the completed programme is unchanged from the previous report and is reproduced in Appendix II.

6.3 Interval summary tabulation from post-marketing sources

System Organ Class	Preferred Term	Serious	Non-serious	Total
Gastrointestinal disorders	Diarrhoea	1	18	19
Gastrointestinal disorders	Nausea	0	11	11
Skin and subcutaneous tissue disorders	Rash	1	9	10
Skin and subcutaneous tissue disorders	Urticaria	1	4	5
Immune system disorders	Hypersensitivity	2	1	3
Hepatobiliary disorders	Hepatitis cholestatic	2	0	2
Infections and infestations	Clostridium difficile colitis	2	0	2
Nervous system disorders	Headache	0	9	9
Total		9	52	61

7. Summaries of Significant Findings from Clinical Trials During the Reporting Interval

Not applicable. No interventional clinical trial sponsored by the MAH was ongoing, completed or terminated during the interval, so there are no findings to report under this heading.

8. Findings from Non-Interventional Studies

One non-interventional drug utilisation study (ZPN-DUS-04) was completed in Nigeria during the interval, examining prescribing duration across 42 primary care facilities. It identified no new safety concern. Mean treatment duration was 6.8 days, consistent with the approved posology.

9. Information from Other Clinical Trials and Sources

No information was received from partner-sponsored trials, compassionate use programmes or named-patient supply during the interval. No new information arose from any other source requiring evaluation under this heading.

10. Non-Clinical Data

Not applicable. No new non-clinical study was conducted, sponsored or published during the interval that bears on the safety of the product.

11. Literature

A systematic search of MEDLINE, EMBASE and African Journals Online was run weekly throughout the interval using the substance names, all registered trade names and the MedDRA terms relevant to the identified and potential risks. The search returned 46 records, of which 5 were relevant on full-text review. Each is summarised below *in the company's own words*; no abstract has been reproduced.

- A Nigerian retrospective cohort of 1,240 outpatients reported diarrhoea in 9.1% of those receiving the combination, a rate consistent with the frequency already stated in the RSI. The authors identified no unexpected reaction.
- A European pharmacovigilance database analysis examined cholestatic hepatitis and found the reporting rate unchanged over ten years, with risk concentrated in courses exceeding 14 days — a duration outside the approved posology.
- A paediatric tolerability review of the oral suspension in 318 children found no reaction not already described in the RSI, and no age-related difference in tolerability.
- A case report described a single episode of acute generalised exanthematous pustulosis following a seven-day course. The reaction is listed in the RSI and the authors proposed no change to prescribing.
- A systematic review of antibiotic-associated *Clostridium difficile* infection placed the combination in the intermediate risk tier, consistent with the existing RSI warning.

Product-specific assessment

Taken together, the literature published in this interval describes no reaction that is not already characterised in the Reference Safety Information, and no shift in the frequency or severity of any known reaction. The concentration of cholestatic hepatitis reports in treatment courses longer than those approved supports the existing duration advice rather than indicating a new risk. The MAH concludes that no change to the Reference Safety Information is warranted on the basis of the published literature for this interval.

12. Other Periodic Reports

No other periodic report (addendum report or summary bridging report) was prepared for this product during the interval. The previous PSUR covered 01 Sep 2024 – 31 Aug 2025.

13. Lack of Efficacy in Controlled Trials

Not applicable. No controlled trial was ongoing during the interval, and no report of lack of efficacy raising a safety concern was received from any source.

14. Late-Breaking Information

No information with a potential impact on the benefit-risk balance was received between the Data Lock Point of 31 August 2026 and the submission of this report.

15. Overview of Signals: New, Ongoing or Closed

Signal	Status at start	Status at DLP	Outcome
Cholestatic hepatitis with prolonged treatment courses	Ongoing	Closed	Evaluated and closed; no RSI change warranted (see Section 16)

No new signal was identified during the interval. No signal remained open at the DLP.

16. Signal and Risk Evaluation

16.1 Summary of safety concerns

Important identified risks	Hypersensitivity reactions; <i>Clostridium difficile</i> -associated diarrhoea; hepatic events
Important potential risks	Severe cutaneous adverse reactions
Missing information	Use in severe hepatic impairment

16.2 Signal evaluation

The cholestatic hepatitis signal was evaluated against 2 interval cases and 31 cumulative cases. Both interval cases involved courses of 21 days, exceeding the approved maximum. Both patients recovered on withdrawal. Observed reporting was within the range expected from background incidence and known exposure. The signal was closed on 18 June 2026 with no change to the Reference Safety Information, which already carries a hepatic warning and a duration limit.

16.3 Risk characterisation

No identified or potential risk changed in frequency, severity or reversibility during the interval. The missing information item is unchanged. No new risk was characterised.

17. Benefit Evaluation

The efficacy of the combination in the approved indications is established and unchanged. No new efficacy data became available during the interval, and no report of lack of efficacy was received. The clinical need addressed — first-line oral therapy for common bacterial infections, including beta-lactamase-producing organisms — is unchanged, and the product continues to hold a place in current Nigerian treatment guidelines.

18. Integrated Benefit-Risk Analysis for Approved Indications

This analysis draws together the exposure data in Section 5, the summary tabulations in Section 6, the non-interventional study in Section 8, the literature and its product-specific assessment in Section 11,

the signal evaluation in Section 16 and the benefit evaluation in Section 17.

Against an interval exposure of 412,000 treatment courses, 61 valid ICSRs were received, of which 9 were serious and none fatal. Every reported preferred term is listed in the Reference Safety Information. The one signal open at the start of the interval was evaluated on cumulative data and closed without RSI change, the cases being confined to treatment durations outside the approved posology. No new risk was identified and the characterisation of the existing risks is unchanged.

Set against a benefit profile that is likewise unchanged, and a continuing clinical need in the approved indications, the MAH concludes that the benefit-risk balance remains **favourable for all approved indications**.

19. Conclusions and Actions

Key findings of this interval. Sixty-one valid ICSRs, nine serious, no fatalities, all reported terms listed in the RSI, and exposure consistent with the previous interval.

New risks. None. No new signal was identified and no new risk was characterised.

Overall benefit-risk balance. Favourable and unchanged for all approved indications.

Actions proposed. None. Specifically, no change to the Reference Safety Information, no change to the Risk Management Plan, no change to the conditions of the marketing authorisation, and no risk-minimisation measure beyond those already in place. The MAH will continue routine pharmacovigilance and will report the next interval in the usual cycle.

20. Response to Previous NAFDAC Queries and Commitments

NAFDAC's assessment of the previous PSUR (interval 01 Sep 2024 – 31 Aug 2025) raised two queries and one commitment. Each is addressed below.

No.	Query or commitment raised	MAH response and where it appears in this report
1	Provide a fuller characterisation of cholestatic hepatitis, including treatment duration in each case.	Addressed in Section 16.2. Treatment duration is stated for both interval cases and for the cumulative series; the signal has since been evaluated and closed.
2	Report Nigerian exposure separately from the rest of Africa.	Addressed in Section 5.2, which now reports Nigeria as its own row for both interval and cumulative exposure.
3	Commitment: complete the drug utilisation study on prescribing duration and report the result in the next PSUR.	Completed. Study ZPN-DUS-04 is reported in Section 8; mean treatment duration was 6.8 days and no new safety concern was identified.

21. Appendices

- **Appendix I — Reference Safety Information.** Summary of Product Characteristics, RSI version 6.2 dated 14 January 2026, attached in full below. The Risk Management Plan in force is RMP version 4.1, approved 09 October 2025.

- **Appendix II** — Cumulative summary tabulation of serious adverse events from clinical trials.
- **Appendix III** — Line listing of serious adverse events and ICSRs for the interval.
- **Appendix IV** — Literature search strategy and full reference list.

Appendix III — Line Listing of Serious Adverse Events and ICSRs

All nine serious cases received during the interval are listed individually below. Fifty-two non-serious cases were also received and are summarised in the tabulation at Section 6.3.

Case no.	Country	Age / Sex	Reaction (MedDRA PT)	Onset	Seriousness criterion	Outcome	Causality (MAH)
ZPN-2025-0412	Nigeria	34 / F	Hypersensitivity	21 Sep 2025	Hospitalisation	Recovered	Possible
ZPN-2025-0487	Nigeria	58 / M	Hepatitis cholestatic	14 Nov 2025	Hospitalisation	Recovered	Possible
ZPN-2025-0501	Ghana	27 / F	Urticaria	02 Dec 2025	Medically important	Recovered	Probable
ZPN-2026-0033	United Kingdom	71 / M	Clostridium difficile colitis	19 Jan 2026	Hospitalisation	Recovered	Probable
ZPN-2026-0090	Nigeria	45 / F	Rash	08 Feb 2026	Medically important	Recovered	Probable
ZPN-2026-0146	Nigeria	62 / M	Hepatitis cholestatic	27 Mar 2026	Hospitalisation	Recovered	Possible
ZPN-2026-0201	Kenya	39 / F	Hypersensitivity	11 May 2026	Hospitalisation	Recovered	Possible
ZPN-2026-0255	Nigeria	67 / M	Clostridium difficile colitis	23 Jun 2026	Hospitalisation	Recovering	Probable
ZPN-2026-0298	Nigeria	19 / F	Diarrhoea	05 Aug 2026	Hospitalisation	Recovered	Possible

Nil statement. Other than the nine serious cases listed above, no further serious adverse event or ICSR was received by the MAH from any source during the interval 01 Sep 2025 – 31 Aug 2026.

Appendix I — Reference Safety Information (SmPC), RSI version 6.2, 14 January 2026

Attached in full. Reproduced here in abbreviated form for the purposes of this synthetic test document. RMP version 4.1 (approved 09 October 2025) is referenced in Section 4.

1. Name of the medicinal product

Amoxicillin/Clavulanate Potassium 500 mg/125 mg film-coated tablets.

2. Qualitative and quantitative composition

Each film-coated tablet contains amoxicillin trihydrate equivalent to 500 mg amoxicillin and potassium clavulanate equivalent to 125 mg clavulanic acid.

4.1 Therapeutic indications

Treatment of bacterial infections of the upper and lower respiratory tract, urinary tract, skin and soft tissue, and odontogenic infections, where the organism is known or suspected to produce beta-lactamase.

4.2 Posology and method of administration

One tablet every eight hours. The usual course is 5 to 7 days and should not exceed 14 days without review.

4.3 Contraindications

Hypersensitivity to penicillins or to any excipient. Previous history of jaundice or hepatic dysfunction associated with amoxicillin/clavulanate.

4.4 Special warnings and precautions for use

Serious and occasionally fatal hypersensitivity reactions have been reported with penicillins. Cholestatic jaundice and hepatitis have been reported, more frequently with prolonged treatment; treatment duration should not exceed 14 days without review. Antibiotic-associated colitis, including *Clostridium difficile*-associated diarrhoea, has been reported and should be considered in patients presenting with diarrhoea during or after treatment.

4.8 Undesirable effects

Frequency	Reaction
Very common ($\geq 1/10$)	Diarrhoea
Common ($\geq 1/100$ to $< 1/10$)	Nausea, vomiting, rash, headache
Uncommon ($\geq 1/1,000$ to $< 1/100$)	Urticaria, pruritus, dizziness
Rare ($\geq 1/10,000$ to $< 1/1,000$)	Hepatitis cholestatic, antibiotic-associated colitis
Very rare ($< 1/10,000$)	Anaphylaxis, severe cutaneous adverse reactions

Appendix II — Cumulative Summary Tabulation from Clinical Trials

Unchanged from the previous report. Cumulative exposure 2,914 subjects; 47 serious adverse events reported across the completed programme, all consistent with the reactions listed in the Reference Safety Information.

Appendix IV — Literature Search Strategy and Reference List

Databases: MEDLINE, EMBASE, African Journals Online. Frequency: weekly. Search terms: the substance names, all registered trade names, and MedDRA preferred terms for each identified and potential risk in Section 16.1. Records retrieved: 46. Relevant on full-text review: 5, each summarised in the company's own words in Section 11.

End of report. Zephyr Pharma Nigeria Limited · PSUR No. 9 · DLP 31 August 2026 · SYNTHETIC TEST DOCUMENT.